

Geriatric Nursing — MCQ Bank

50 Multiple Choice Questions with Answers | 5 Lectures

Lecture 1 — Nutrition in the Elderly

1. Compared to younger adults, malnutrition is:

- A. Less common in elderly
- B. More common in elderly
- C. Equally common in both groups
- D. Rare in elderly

✓ Answer: B — More common in elderly

Malnutrition is more prevalent in older adults due to physiological, psychological, and social changes that affect nutritional intake.

2. The percentage of body water in an elderly person compared to an adult is:

- A. 70% in elderly vs 50% in adults
- B. 50% in elderly vs 70% in adults
- C. Equal at 60% in both
- D. 80% in elderly vs 60% in adults

✓ Answer: B — 50% in elderly vs 70% in adults

With aging, total body water decreases from about 70% in adults to approximately 50% in the elderly, increasing the risk of dehydration.

3. The recommended daily fiber intake for the elderly is:

- A. 10–15 g/day
- B. 15–20 g/day
- C. 25–30 g/day
- D. 40–50 g/day

✓ Answer: C — 25–30 g/day

A daily fiber intake of 25–30 grams helps prevent constipation, regulate blood sugar, lower cholesterol, and reduce the risk of intestinal cancer.

4. The main reason for decreased Basal Metabolic Rate (BMR) in the elderly is:

- A. Increased muscle mass
- B. Reduced muscle mass and increased fatty tissue
- C. Increased physical activity
- D. Improved nutrient absorption

✓ Answer: B — Reduced muscle mass and increased fatty tissue

BMR decreases by 15–20% in older adults due to loss of metabolically active muscle mass and increased body fat percentage.

5. The safe daily protein intake for an elderly person is:

- A. 0.5 g/kg body weight
- B. 1.0 g/kg body weight
- C. 2.0 g/kg body weight
- D. 3.0 g/kg body weight

✓ Answer: B — 1.0 g/kg body weight

An intake of 1.0 g/kg/day is considered safe for older adults to compensate for decreased protein stores and maintain protein synthesis.

6. Deficiency of which vitamins is most associated with cognitive impairment in the elderly?

- A. Vitamin A only
- B. Vitamin C only
- C. Vitamin B12, B6, and Folate
- D. Vitamin K only

✓ Answer: C — Vitamin B12, B6, and Folate

Malnutrition particularly involving vitamins B12, B6, and folate is associated with cognitive impairment in the elderly.

7. The most widely used nutritional screening tool for the elderly is:

- A. BMI alone
- B. Mini-Nutritional Assessment (MNA)
- C. SCALES assessment alone
- D. Determine Checklist alone

✓ Answer: B — Mini-Nutritional Assessment (MNA)

The MNA is the most widely used tool; a score of 17–23.5 indicates risk of malnutrition, and a score below 17 indicates malnourishment.

8. Which of the following is NOT a socio-psychological cause of malnutrition in the elderly?

- A. Loneliness and social isolation
- B. Depression and anxiety
- C. Loss of independence
- D. Increased physical activity

✓ Answer: D — Increased physical activity

Increased physical activity is not a cause of malnutrition. Socio-psychological causes include loneliness, depression, loss of self-esteem, and loss of independence.

9. The recommended daily calcium intake for the elderly is:

- A. 400 mg/day
- B. 600 mg/day
- C. 800 mg/day
- D. 1200 mg/day

✓ Answer: C — 800 mg/day

Elderly individuals need 800 mg of calcium per day to compensate for age-related bone loss, improve calcium balance, and reduce fracture risk.

10. Which dietary tip is most appropriate to improve nutritional intake in the elderly?

- A. Serve 3 large meals per day
- B. Serve 6 small meals per day
- C. Avoid soft foods
- D. Reduce fluids to prevent urination

✓ Answer: B — Serve 6 small meals per day

Serving six small meals instead of three large ones is recommended because large portions can look overwhelming and reduce appetite in the elderly.

Lecture 2 — Falls Prevention in the Elderly

11. According to the WHO, a fall is defined as:

- A. Any accident resulting in a fracture
- B. An unexpected event in which a person comes to rest on the ground or lower level
- C. Sudden loss of consciousness only
- D. Slipping inside the bathroom only

✓ Answer: B — An unexpected event in which a person comes to rest on the ground or lower level

The WHO defines a fall as an unexpected event resulting in the person resting on the ground, with or without loss of consciousness or injury.

12. How many individuals die from falls globally each year?

- A. 100,000
- B. 300,000
- C. 646,000

D. 1,000,000

✓ Answer: C — 646,000

An estimated 646,000 individuals die from falls globally each year, over 80% of whom are in low- and middle-income countries.

13. Which of the following is an extrinsic (environmental) risk factor for falls?

- A. Muscle weakness
- B. Visual impairment
- C. Absence of stair handrails
- D. Depression and anxiety

✓ Answer: C — Absence of stair handrails

Extrinsic factors are environmental, including lack of stair handrails, poor stair design, dim lighting, slippery surfaces, and obstacles.

14. What percentage of hip fractures are associated with falls in persons above 70 years?

- A. 50%
- B. 70%
- C. 80%
- D. 90%

✓ Answer: D — 90%

Ninety percent of hip fractures are associated with falls in persons above 70 years of age.

15. Which exercise is recommended to improve balance and reduce fear of falling in the elderly?

- A. Heavy weightlifting
- B. Tai Chi
- C. Fast swimming
- D. Sprinting

✓ Answer: B — Tai Chi

Tai Chi is recommended as it provides both physical benefits (balance, strength) and psychological support to reduce fall anxiety.

16. To prevent postural hypotension in the elderly, the nurse should instruct the patient to:

- A. Stand up quickly from bed in the morning
- B. Sit on the side of the bed with feet dangling before standing
- C. Avoid drinking fluids
- D. Wear high-heeled shoes

✓ Answer: B — Sit on the side of the bed with feet dangling before standing
Postural hypotension increases with age, especially with diuretic or antihypertensive medications, so gradual position changes are essential.

17. Which tool is used to assess fall risk in the elderly by measuring mobility?

- A. Mini-Mental State Examination
- B. Timed Up and Go Test
- C. Braden Scale
- D. MNA Score

✓ Answer: B — Timed Up and Go Test
The Timed Up and Go (TUG) Test measures mobility by timing how long it takes to stand, walk a short distance, and return to the chair.

18. Which bathroom modification is correct to prevent falls?

- A. Use smooth tiles in the bathroom
- B. Install raised toilet seat and nonskid mat on tub floors
- C. Remove night lighting
- D. Narrow bathroom pathways

✓ Answer: B — Install raised toilet seat and nonskid mat on tub floors
Bathroom modifications should include elevated toilet seat, nonskid mats, and handrails on both sides to reduce fall risk.

19. Fear of falling again in the elderly leads to:

- A. Increased physical activity
- B. Improved balance
- C. Reduced physical activity and increased social isolation
- D. Improved social relationships

✓ Answer: C — Reduced physical activity and increased social isolation
Fear of falling can reduce physical activity and social engagement, which may further weaken strength and balance, heightening risk of future falls.

20. Which of the following is an intrinsic (personal) risk factor for falls?

- A. Dim lighting
- B. Slippery floors
- C. Use of sedatives, antihypertensives, or diuretics
- D. Loose rugs

✓ Answer: C — Use of sedatives, antihypertensives, or diuretics
Medications such as sedatives, antihypertensives, and diuretics are intrinsic risk factors as they cause side effects like dizziness and orthostatic hypotension.

Lecture 3 — Dementia Disease

21. What is the most common cause of dementia?

- A. Vascular dementia
- B. Alzheimer's disease
- C. Lewy body dementia
- D. Frontotemporal dementia

✓ Answer: B — Alzheimer's disease

Alzheimer's disease accounts for approximately two-thirds of all dementia cases and is the most common form.

22. In Alzheimer's disease, plaques and tangles refer to:

- A. Plaques: tau protein between cells; Tangles: amyloid inside cells
- B. Plaques: amyloid protein deposits between nerve cells; Tangles: tau protein deposits inside nerve cells
- C. Both are the same protein
- D. Neither is related to nerve cells

✓ Answer: B — Plaques: amyloid protein between nerve cells; Tangles: tau protein inside nerve cells

Both plaques and tangles stop communication between nerve cells and cause irreversible neuronal death.

23. Which of the following is a reversible (treatable) cause of dementia?

- A. Alzheimer's disease
- B. Vascular dementia
- C. Vitamin B12 deficiency
- D. Lewy body dementia

✓ Answer: C — Vitamin B12 deficiency

Nutritional deficiencies such as vitamin B12 and thiamine deficiency are reversible causes of dementia that can be treated.

24. The cognitive impairment cut-off score in the MMSE test is:

- A. Below 28
- B. Below 26
- C. Below 24
- D. Below 20

✓ Answer: C — Below 24

The Mini-Mental State Examination (MMSE) has a maximum score of 30; a score below 24 typically indicates cognitive impairment.

25. Which non-pharmacological intervention uses personal memories, music, and familiar objects to improve mood in dementia patients?

- A. Cognitive Stimulation Therapy
- B. Reminiscence Therapy
- C. Validation Therapy
- D. Virtual Reality Therapy

✓ Answer: B — Reminiscence Therapy

Reminiscence Therapy uses personal history items such as photos, music, and familiar objects to evoke memories, improve mood, reduce agitation, and strengthen patient-family relationships.

26. Which risk factor for dementia is modifiable?

- A. Age
- B. Female sex
- C. Family genetic history
- D. High blood pressure

✓ Answer: D — High blood pressure

Modifiable risk factors include cardiovascular conditions like hypertension, high cholesterol, diabetes, obesity, smoking, and physical inactivity.

27. What is the correct non-pharmacological approach for managing aggressive behavior in a dementia patient?

- A. Restrain the patient immediately
- B. Use distraction and redirect attention to a different activity
- C. Raise voice to gain attention
- D. Completely ignore the behavior

✓ Answer: B — Use distraction and redirect attention to a different activity

Distraction techniques redirect attention to a different activity when the patient becomes agitated, and validation acknowledges their emotions.

28. Why is early detection of dementia important?

- A. No benefit since there is no treatment
- B. It allows timely intervention and improved quality of life
- C. It only increases family anxiety
- D. Used only for research purposes

✓ Answer: B — It allows timely intervention and improved quality of life
Early identification allows for timely interventions, better care planning, and can help delay the progression of symptoms through early management.

29. Which technology is used to monitor a dementia patient at risk of wandering?

- A. Augmented reality devices
- B. GPS trackers
- C. Social robots only
- D. X-ray monitoring

✓ Answer: B — GPS trackers
GPS trackers help monitor patients who are at risk of wandering, while smart home systems use sensors to monitor patient safety.

30. Which of the following is a symptom of dementia?

- A. Improved memory function
- B. Difficulty finding words and disorientation in time and place
- C. Increased social engagement
- D. Improved decision-making ability

✓ Answer: B — Difficulty finding words and disorientation in time and place
Dementia symptoms include memory loss, language difficulties, disorientation, changes in mood and behavior, and difficulty with daily activities.

Lecture 4 — Pressure Ulcer

31. A pressure ulcer is best defined as:

- A. A skin inflammation caused by heat
- B. An injury to the skin and/or underlying tissue from pressure or shear, usually over a bony prominence
- C. A wound caused by bacterial infection only
- D. Ulcers resulting from medication side effects

✓ Answer: B — An injury to the skin and/or underlying tissue from pressure or shear, usually over a bony prominence
Persistent pressure disrupts blood flow, causing ischemia, anoxia, and eventual cell death.

32. What percentage of all pressure ulcers occur in elderly patients?

- A. 30%
- B. 50%
- C. 70%
- D. 90%

✓ Answer: C — 70%

Older people account for 70% of all pressure ulcers, with the highest incidence in hospitalized elderly undergoing orthopedic procedures.

33. In which stage of pressure ulcer does the epidermis remain intact?

- A. Stage II
- B. Stage III
- C. Stage I only
- D. Stage IV

✓ Answer: C — Stage I only

In Stage I, there is erythema that does not resolve within 30 minutes of pressure relief, but the epidermis remains intact. This stage is reversible with intervention.

34. In Stage IV pressure ulcer, tissue destruction extends to:

- A. Epidermis only
- B. Skin and dermis
- C. Subcutaneous tissue
- D. Muscles, bones, and joints

✓ Answer: D — Muscles, bones, and joints

Stage IV involves deep tissue destruction extending through subcutaneous tissue to fascia, possibly involving muscle layers, joints, and/or bone.

35. The primary risk factor for pressure ulcer development is:

- A. Malnutrition only
- B. Infections
- C. Immobility and unrelieved pressure
- D. Dehydration only

✓ Answer: C — Immobility and unrelieved pressure

The primary risk factors are immobility and limited activity with positioning that exerts unrelieved pressure on tissue confined between non-pliable surfaces.

36. Which factors affect tissue tolerance in pressure ulcer development?

- A. Skin color only
- B. Moisture, friction, shear force, nutritional status, age, and arterial pressure
- C. Type of mattress only
- D. Patient's weight only

✓ Answer: B — Moisture, friction, shear force, nutritional status, age, and arterial pressure

Tissue tolerance refers to the ability of tissue to distribute and compensate for pressure; it is affected by multiple factors beyond pressure alone.

37. Why is regular repositioning of immobile patients important in pressure ulcer prevention?

- A. To improve breathing only
- B. To restore blood circulation to tissues over bony prominences
- C. To reduce pain only
- D. To improve psychological status

✓ Answer: B — To restore blood circulation to tissues over bony prominences
Regular repositioning displaces body weight from pressure points, facilitating circulation to the skin and preventing ischemia.

38. Which substance should be avoided in pressure ulcer wound care?

- A. Moist dressings
- B. Povidone-iodine (Betadine)
- C. Normal saline solutions
- D. Collagen dressings

✓ Answer: B — Povidone-iodine (Betadine)
Betadine is a caustic product that should NOT be applied to tissue, as it impairs healing. The wound should be kept clean and moist.

39. Which laboratory test is most important for monitoring nutritional status in pressure ulcer patients?

- A. Complete blood count only
- B. Serum albumin, hematocrit, and hemoglobin
- C. Kidney function tests only
- D. Blood glucose levels only

✓ Answer: B — Serum albumin, hematocrit, and hemoglobin
Nutritional intake should be monitored along with serum albumin, hematocrit, and hemoglobin; supplementation should be considered if deficiencies exist.

40. The primary nursing role in primary prevention of pressure ulcers is to:

- A. Wait for erythema to appear before intervening
- B. Reduce friction, minimize moisture, reposition patients, and redistribute body weight
- C. Apply topical antibiotics routinely
- D. Use compression bandages always

✓ Answer: B — Reduce friction, minimize moisture, reposition patients, and redistribute body weight
Nurses focus on prevention by eliminating friction, reducing moisture to prevent maceration, and displacing body weight from bony prominences.

Lecture 5 — Urinary Incontinence

41. Urinary incontinence is defined as:

- A. Frequent urge to urinate only
- B. Any involuntary leakage of urine
- C. Complete kidney failure to produce urine
- D. Total urinary retention

✓ Answer: B — Any involuntary leakage of urine

Urinary incontinence is any involuntary leakage of urine. It is a common and distressing problem that may have a profound impact on quality of life.

42. Which type of urinary incontinence occurs with pressure such as coughing, sneezing, or laughing?

- A. Urge incontinence
- B. Overflow incontinence
- C. Stress incontinence
- D. Functional incontinence

✓ Answer: C — Stress incontinence

Stress incontinence is due to insufficient strength of pelvic floor muscles; urine leaks when pressure is exerted on the bladder.

43. The most common cause of urinary incontinence in men over 40 years is:

- A. Bladder infection
- B. Enlarged prostate (BPH)
- C. Severe constipation
- D. Sphincter muscle weakness

✓ Answer: B — Enlarged prostate (BPH)

Enlarged prostate is the most common cause of incontinence in men after the age of 40.

44. Which type of urinary incontinence results from the bladder not emptying completely?

- A. Stress incontinence
- B. Urge incontinence
- C. Overflow incontinence
- D. Structural incontinence

✓ Answer: C — Overflow incontinence

Overflow incontinence is urine leakage with or without feeling the need to void, due to a bladder that does not empty completely.

45. The first-line nursing intervention for stress urinary incontinence is:

- A. Permanent urinary catheterization
- B. Teaching pelvic floor muscle exercises (Kegel exercises)
- C. Administering diuretics
- D. Restricting all fluids

✓ Answer: B — Teaching pelvic floor muscle exercises (Kegel exercises)
Pelvic floor muscle exercises are the first-line intervention for stress incontinence as they strengthen the muscles supporting bladder control.

46. Why should indwelling catheters be avoided in elderly patients with urinary incontinence whenever possible?

- A. Because they are too painful
- B. Because they significantly increase the risk of urinary tract infections
- C. Because they cause bladder obstruction
- D. Because they do not work in the elderly

✓ Answer: B — Because they significantly increase the risk of urinary tract infections
Indwelling urinary catheters should be avoided whenever possible to avoid risk for urinary tract infection. Long-term use significantly increases infection risk.

47. What is the social and psychological impact of urinary incontinence on the elderly?

- A. No effect on social life
- B. Social withdrawal, embarrassment, and reduced quality of life
- C. Improved social relationships
- D. Increased self-confidence

✓ Answer: B — Social withdrawal, embarrassment, and reduced quality of life
Urinary incontinence can affect social, work, and personal relationships, leading to emotional distress and social isolation.

48. What environmental modification helps an elderly patient with urinary incontinence maintain continence?

- A. Reducing corridor lighting
- B. Providing a clear path to the bathroom with night lighting and grab bars
- C. Removing all seats from corridors
- D. Keeping the bathroom door closed at all times

✓ Answer: B — Providing a clear path to the bathroom with night lighting and grab bars
Environmental modifications include ensuring a clear path to the bathroom, installing night lights, grab bars, and using raised toilet seats.

49. A bladder diary includes documentation of:

- A. Medications used only

- B. Voiding times, volumes, leakage episodes, and fluid intake for 3–7 days
- C. Urinalysis results only
- D. History of chronic diseases only

✓ Answer: B — Voiding times, volumes, leakage episodes, and fluid intake for 3–7 days
The bladder diary is a key assessment tool in which patients log voiding times, volumes, leakage episodes, and fluid intake for 3–7 days.

50. Which statement is correct regarding urinary incontinence and aging?

- A. Urinary incontinence is an inevitable result of aging and cannot be treated
- B. Urinary incontinence is more common with aging but is treatable and not inevitable
- C. Urinary incontinence always affects men more than women
- D. Urinary incontinence has no relationship to medications

✓ Answer: B — Urinary incontinence is more common with aging but is treatable and not inevitable
Although it occurs more often as people get older, urinary incontinence is NOT an inevitable consequence of aging and is almost always treatable.

51-Which change commonly increases the risk of falls in older adults?

- A. Improved reflexes
- B. Increased bone density
- C. Impaired balance and gait
- D. Enhanced muscle tone

52- Which symptom is commonly seen in early Alzheimer's disease?

- A. Sudden paralysis
- B. Short-term memory loss
- C. Severe tremors
- D. Loss of hearing

53- The nurse caring for a patient with dementia should primarily:

- A. Frequently change routines
- B. Encourage complete bed rest
- C. Maintain a calm and familiar environment
- D. Avoid communication with the patient

54- Older adults are at increased risk of malnutrition because of:

- A. Increased digestive efficiency
- B. Improved appetite
- C. Difficulty chewing and swallowing
- D. Increased metabolic demands

55-A patient who experiences sudden urgency to urinate followed by

involuntary leakage most likely has:

- A. Stress incontinence
- B. Overflow incontinence
- C. Functional incontinence
- D. Urge incontinence

56- Which area is most prone to development of pressure ulcers in bedridden patients?

- A. Abdomen
- B. Sacrum
- C. Forearm
- D. Neck

57--Osteoporosis is best described as:

- A. Joint inflammation
- B. Muscle degeneration
- C. Loss of bone density
- D. Cartilage hypertroph

58- Dehydration in elderly patients is commonly due to:

- A. Excessive thirst
- B. Reduced kidney function
- C. Increased fluid intake

D. Increased sweating

59- Which factor may contribute to depression in older adults?

A. Strong social support

B. Chronic illness and isolation

C. Regular physical activity

D. Adequate sleep

60- Osteoporosis increases the risk of:

A. Hypertension

B. Bone fractures

C. Lung infections

D. Kidney stones

61- Which sign may indicate malnutrition in an older adult?

A. Weight gain

B. Increased muscle mass

C. Unintentional weight loss

D. Increased appetite

62- Which vitamin is essential for calcium absorption in older adults?

A. Vitamin A

B. Vitamin D

C. Vitamin K

D. Vitamin C

64. Which age-related change contributes most to falls?

A. Improved vision

B. Increased muscle strength

C. Slower reaction time

D. Increased flexibility

65. Which assistive device may help reduce fall risk in older adults with impaired mobility?

A. Oxygen mask

B. Walking cane

C. Urinary catheter

D. Feeding tube

66. Which medication group is most associated with increased fall risk?

- A. Vitamins
- B. Sedatives and hypnotics**
- C. Antacids
- D. Antibiotics

67. Which symptom is commonly observed in moderate dementia?

- A. Complete independence
- B. Difficulty performing daily activities**
- C. Enhanced memory
- D. Increased concentration

68. Which communication strategy is most appropriate for a patient with dementia?

- A. Speak rapidly
- B. Use short simple sentences**
- C. Ask multiple questions at once
- D. Correct every mistake immediately

69. Wandering behavior in dementia increases the risk of:

- A. Malnutrition only
- B. Falls and injury**
- C. Hypertension
- D. Osteoporosis

70. Which patient is at highest risk for pressure ulcer development?

- A. Ambulatory older adult
- B. Bedridden patient with poor nutrition**
- C. Patient with good mobility
- D. Patient with normal hydration

71. Which device helps redistribute pressure and prevent pressure ulcers?

- A. Hard mattress
- B. Pressure-relieving mattress**
- C. Wheelchair cushion only
- D. Sandbag

72. A Stage II pressure ulcer is characterized by:

- A. Intact skin with redness only
- B. Partial-thickness skin loss**
- C. Exposure of muscle and bone
- D. Necrosis of deep tissues

73. Functional incontinence occurs when the patient:

- A. Cannot reach the toilet in time due to physical or cognitive limitations
- B. Has bladder spasms only
- C. Has enlarged prostate only
- D. Has urinary infection only

74. Which assessment tool is useful in evaluating urinary incontinence?

- A. Glasgow Coma Scale
- B. Bladder diary
- C. Braden Scale
- D. MMSE

75. Polypharmacy refers to:

- A. Taking one medication daily
- B. Use of multiple medications simultaneously
- C. Refusal to take medications
- D. Use of herbal remedies only

76. Which sensory change is commonly associated with aging?

- A. Improved hearing
- B. Increased visual acuity
- C. Reduced hearing ability (presbycusis)
- D. Enhanced night vision

77. Which factor promotes healthy aging?

- A. Physical inactivity
- B. Social isolation
- C. Regular exercise and social engagement
- D. Smoking

78. An elderly patient is at increased risk of adverse drug reactions because of:

- A. Faster drug metabolism
- B. Age-related changes in liver and kidney function
- C. Increased muscle mass
- D. Enhanced absorption of all drugs

79. Which intervention helps maintain independence in older adults?

- A. Doing all activities for the patient
- B. Encouraging participation in self-care activities
- C. Restricting mobility
- D. Limiting decision-making

80. The primary goal of geriatric nursing is to:

A. Cure all chronic diseases

B. Promote optimal function, independence, and quality of life

C. Prevent aging

D. Eliminate all medications